HERNIA LOUNGE · RECORDING SCRIPT

# Igor Belyansky on Posterior Rectus Sheath Reloading

Two parts · 22 short clips · Part 1: 9 clips, about 8 min · Part 2: 13 clips, about 9 min

## How this works

Each question becomes one separate video. Record them one at a time and stop between clips. Pedro will edit each clip and overlay the question in a text box on screen, so you do not need to read the question aloud. Start each answer directly, as if someone had just asked you across the table.

**Your words, your way.** The guide lines under each question are the points you already made in the AWR Surgeons Community thread. They are there so you do not have to remember what you said, not to be read. Answer in your own words and keep it tight: there are many questions, and short direct answers cut better than complete ones.

**If you stumble:** do not stop the recording. Just freeze, stay still and silent for about three seconds, then start the answer again from the top. We edit the rest out.

**Two parts.** Part 1 is the concept (why an intact repair can still bulge). Part 2 is the practice (technique, selection, risks). Part 1 ends with a one-line teaser for Part 2; it is marked in the script. You can record both parts in one sitting.

## Setup checklist

- **Location:** your backyard. Sit or stand with the garden behind you, some depth between you and the background.
- **Light:** daylight from the front or slightly to one side, never behind you. Early morning or late afternoon is best. Avoid direct midday sun and moving shadows.
- **Sound:** a lapel or shotgun microphone, not the built-in mic. Watch for wind, lawn equipment and birds close by; a windscreen on the mic helps outdoors.
- **Framing:** horizontal (landscape), camera at eye level, head and shoulders with a little room above the head. Keep the top third of the frame clear, the question box goes there.
- **Consistency:** same clothes, same spot, same light for the whole session so the clips cut together. Look straight into the lens.
- **Pace:** two seconds of silence before you start and after you finish each answer, so Pedro has room to cut.
- **Naming:** if you can, name each file with the clip number (P1-03, P2-07). Otherwise record in order and Pedro will match them.

■ Question as asked in the thread ■ Adapted from your own statements ■ New question, answer freely

PART 1

# The concept — why an intact repair can still bulge

9 clips · about 8 minutes

## THE PROBLEM

CLIP P1-01 target ~60 s

### What is the problem you are trying to address, and why is it not the same as recurrence?

**Guide, from what you said in the thread:**

- Patients with an intact repair, no recurrence, who still bulge or look like they have a diastasis
- Cause: posterior layer bridged with peritoneum, posterior forces never reloaded
- This is how eTEP was described in 2015 and how you operated for years; your understanding has changed

CLIP P1-02 target ~40 s

### Why does this matter so much in diastasis patients?

**Guide, from what you said in the thread:**

- They expect contour, not only a repair; an intact repair that still bulges fails that expectation
- The point is not how anyone should operate, it is aligning what we promise with what the patient will see

## MECHANISM AND DATA

CLIP P1-03 target ~50 s

### What actually happens to the rectus when the posterior layer is bridged?

**Guide, from what you said in the thread:**

- Forces from posterior lamella of internal oblique and transversus stop reaching the midline
- Rectus goes from oval to flat; semilunar line flattens and lateralizes
- Happens even with a perfect anterior closure and mesh in place

CLIP P1-04 target ~50 s

### What does your CT data show, reloaded versus bridged?

**Guide, from what you said in the thread:**

- Reloading does not restore native shape, the rectus still widens, but much less
- Reloading centralizes the two recti; they behave more like one unit
- Bridged: much more widening and lateralization, remarkably similar to post-TAR CT

CLIP P1-05 target ~35 s

### Does reloading improve function, or only what we see on CT?

**Guide, from what you said in the thread:**

- You believe both, but the data show morphology only; no functional claim yet
- The field first has to agree on how to measure abdominal wall function

CLIP P1-06 target ~45 s

### Could this bulging be mesh weight, muscle quality, or something else?

**Guide, from what you said in the thread:**

- Many causes exist (muscle quality, denervation, rehab, visceral fat), each a different mechanism
- You are talking about a technically correct operation with no nerve injury
- Mesh weight is not it: medium weight macroporous polypropylene for years, heavier mesh would not fix this

## THE TAR DISTINCTION

CLIP P1-07 target ~70 s

### Is closing the PRS after a TAR the same thing you are describing?

**Guide, from what you said in the thread:**

- No. After TAR the transversus forces are already released; closing the PRS then is only a visceral barrier, no function
- You are describing reloading WITHOUT TAR, with the sheath still connected to internal oblique and transversus
- Same sheath, completely different biomechanics; that is why unreloaded PRS and TAR look alike on CT

CLIP P1-08 target ~55 s

### So are you against TAR?

**Guide, from what you said in the thread:**

- No, you still do many. In a TAR you knowingly trade some native function for a reconstructed wall and quality of life
- Loss of domain is the perfect example of a trade worth making; a small defect is not
- You are not advocating a TAR just to close the PRS, that would defeat the purpose

## TENSION

CLIP P1-09 target ~55 s

## Do you still believe in a tension-free repair?

**Guide, from what you said in the thread:**

- You are not sure what tension-free even means; every reconstruction has forces on it
- Tension is not the enemy; for function, appropriate tension may be essential
- How much is right? Nobody can say scientifically yet; for now it is an art based on experience

**END OF PART 1. After this answer, pause, then add one line straight to camera, something like: “How I actually put that tension back into the posterior layer, and what can go wrong, that is part two.”**

PART 2

# The practice — technique, selection and risks

13 clips · about 9 minutes

## TECHNIQUE

CLIP P2-01 target ~45 s

### Early on you bridged with peritoneum or the sac. What did that teach you?

**Guide, from what you said in the thread:**

- Excellent barrier, but it does not reload the forces; several hundred early cases
- CT changes were clear and much worse than with reloading
- Bridged patients should be told before surgery they may still bulge

CLIP P2-02 target ~55 s

### Since you started reloading the posterior sheath, has the bulging gone away?

**Guide, from what you said in the thread:**

- Decreased dramatically as the technique evolved
- In some patients you now plicate the posterior layer, not just approximate it, with tension distributed along the whole line
- You see the recti roll medially and the contour change on the table, especially laterally

CLIP P2-03 target ~40 s

### How do you decide who gets posterior plication rather than simple approximation?

**No guide, new question:**

*Not in the thread. Answer from your practice, briefly.*

CLIP P2-04 target ~35 s

### What sutures do you use, and why permanent?

**Guide, from what you said in the thread:**

- Anterior midline plication with permanent suture; absorbable loses the correction as it goes
- Posterior closure reinforced with permanent 2-0 Prolene; mesh center fixed to the PRS with permanent sutures

CLIP P2-05 target ~35 s

### Does the platform matter: robotic, laparoscopic, open?

**Guide, from what you said in the thread:**

- Robotic platform matters to you: you could not reproduce the same high tension, precisely distributed closure laparoscopically
- Open: the same principles are reproducible

## PATIENT SELECTION

CLIP P2-06 target ~55 s

### Is there a defect size beyond which you cannot reload the PRS, and who should not go to eTEP?

**Guide, from what you said in the thread:**

- A width cutoff gets too far into the weeds; many factors decide the operation
- Rule of thumb: retrorectus for longer incisional defects narrower than the retrorectus space at its widest
- Broader message: don't be too eager with eTEP for small and medium hernias

CLIP P2-07 target ~35 s

### Should we be moving toward TAPP or PeTEP so the sheath is never divided?

**Guide, from what you said in the thread:**

- Reasonable for the right cases; you still do many preperitoneal repairs for exactly this reason
- Preperitoneal or IPOM preserves the posterior forces with no retrorectus dissection

## RISKS AND REPRODUCIBILITY

CLIP P2-08 target ~50 s

### What is the main risk of reattaching the PRS?

**Guide, from what you said in the thread:**

- Largely technical: nerve injury in the upper abdomen, where nerves run medially (García-Ureña)
- You cheat the dissection medially up there; no significant denervation seen
- Separate the risk of reattachment itself from the risk of how it is done

CLIP P2-09 target ~30 s

### Have you seen the reloaded sheath disrupt?

**Guide, from what you said in the thread:**

- Extremely rare; recent disruptions were through the peritoneal component, not the reloaded fascia
- Not seen in years, which does not mean it cannot happen

CLIP P2-10 target ~35 s

### How do you follow these patients, and do you get a postoperative CT?

**No guide, new question:**

*Not in the thread. Describe your follow-up and when you scan.*

CLIP P2-11 target ~45 s

### Can a less experienced surgeon reproduce PRS closure under tension safely?

**Guide, from what you said in the thread:**

- You share the concern; that is why you hesitate to promote aggressive plication broadly
- Distributing tension along the whole line, not just pulling edges together, is everything
- Done aggressively without that understanding: more posterior failures and interparietal hernias

## CLOSING

CLIP P2-12 target ~35 s

### What study would settle this?

**No guide, new question:**

*Not in the thread. One or two sentences on what you would want measured.*

CLIP P2-13 target ~45 s

### What is your overall message to surgeons using the retrorectus space?

**Guide, from what you said in the thread:**

- Pull back and reconsider some of the principles we were taught
- Concern: overuse of the retrorectus space without appreciating what not reloading does to morphology and function
- You do not have all the answers, but there is something here that deserves more study

**END OF PART 2.**

FOR THE EDITOR

# Clip list and on-screen question text

One row per clip. Third column is the exact text for the question box. Part 1 ends with a spoken teaser for Part 2 (clip P1-09).

| Clip | Block | Question box text | ~s |
|---|---|---|---|
| P1-01 | The problem | What is the problem you are trying to address, and why is it not the same as recurrence? | 60 |
| P1-02 | The problem | Why does this matter so much in diastasis patients? | 40 |
| P1-03 | Mechanism and data | What actually happens to the rectus when the posterior layer is bridged? | 50 |
| P1-04 | Mechanism and data | What does your CT data show, reloaded versus bridged? | 50 |
| P1-05 | Mechanism and data | Does reloading improve function, or only what we see on CT? | 35 |
| P1-06 | Mechanism and data | Could this bulging be mesh weight, muscle quality, or something else? | 45 |
| P1-07 | The TAR distinction | Is closing the PRS after a TAR the same thing you are describing? | 70 |
| P1-08 | The TAR distinction | So are you against TAR? | 55 |
| P1-09 | Tension | Do you still believe in a tension-free repair? | 55 |
| P2-01 | Technique | Early on you bridged with peritoneum or the sac. What did that teach you? | 45 |
| P2-02 | Technique | Since you started reloading the posterior sheath, has the bulging gone away? | 55 |
| P2-03 | Technique | How do you decide who gets posterior plication rather than simple approximation? | 40 |
| P2-04 | Technique | What sutures do you use, and why permanent? | 35 |
| P2-05 | Technique | Does the platform matter: robotic, laparoscopic, open? | 35 |
| P2-06 | Patient selection | Is there a defect size beyond which you cannot reload the PRS, and who should not go to eTEP? | 55 |
| P2-07 | Patient selection | Should we be moving toward TAPP or PeTEP so the sheath is never divided? | 35 |
| P2-08 | Risks and reproducibility | What is the main risk of reattaching the PRS? | 50 |
| P2-09 | Risks and reproducibility | Have you seen the reloaded sheath disrupt? | 30 |
| P2-10 | Risks and reproducibility | How do you follow these patients, and do you get a postoperative CT? | 35 |
| P2-11 | Risks and reproducibility | Can a less experienced surgeon reproduce PRS closure under tension safely? | 45 |
| P2-12 | Closing | What study would settle this? | 35 |
| P2-13 | Closing | What is your overall message to surgeons using the retrorectus space? | 45 |